

Public Health Emergency Operations Center
"COUSP DRC"

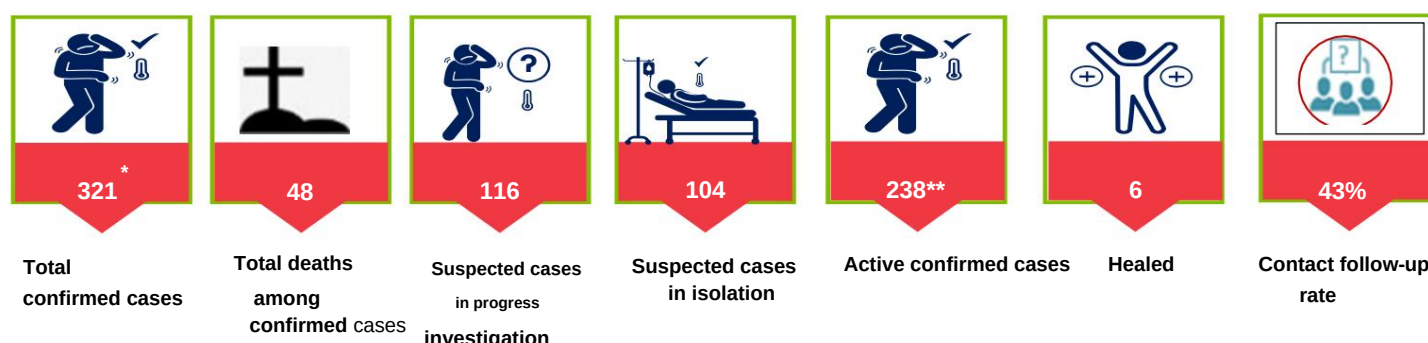
MVE-17 Incident Management System

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Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°017/MVB_31/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (23)	ITURI : Aru, Aungba, Bambu, Damas, Bunia, Gety, Kilo, Komanda, Lita, Mangala, Mongbwalu, Nizi, Nyankunde, Rwampara and Logo NORTH KIVU : Butembo, Goma, Oicha, Kalunguta, Katwa, Kyondo and Beni SOUTH KIVU : Miti-Murhesa
Reporting date	May 31, 2026
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1. HIGHLIGHTS

- **Twelve (12) new confirmed cases** notified on May 31, 2026, distributed in the health zones of Rwampara (5), Bunia (4), Nyankunde (1), Logo (1) and Nizi (1).
- **The Logo health zone is newly affected**, bringing the number of affected health zones to **15 out of 36 (41.7%)** attacks in the province of Ituri.
- **Discharge of four (4) recovered patients** from the Evangelical Medical Center of Bunia, ceremony presided over by the Director General of INSP in the presence of the Director General of WHO.
- **Official inauguration of the newly rehabilitated and handed-over Ebola Treatment Centre (ETC) of CME Rwampara** by the WHO Director-General to the MVE Incident Manager representing the Minister of Public Health.
- **Receipt of 162 units of blood** for transfusion care as part of continuity of essential health care services.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million inhabitants, including more than one million internally displaced persons.

The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting

migrant workers. In September 2018, the province of Ituri had already been affected by the 10th Ebola outbreak which was then raging in North Kivu.

The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.



Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

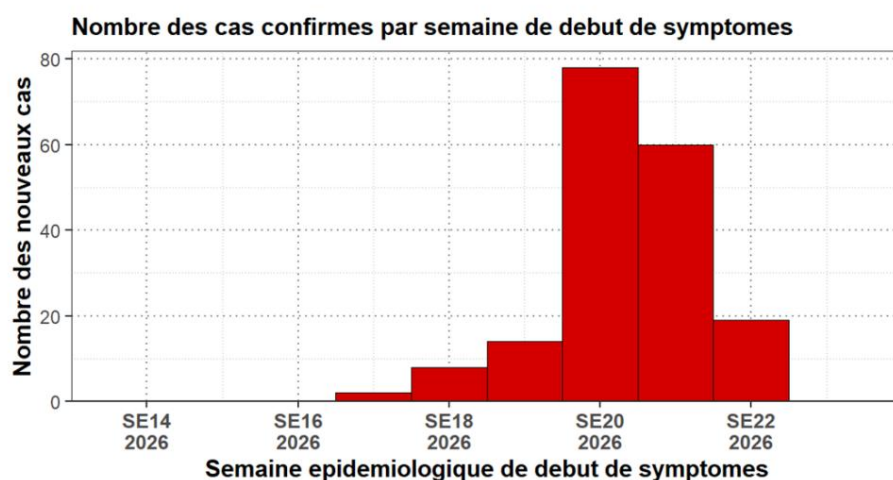
3.1 Spatial Distribution

Province	Case confirmed	Death (confirmed)	CFR	Health zones touched	New cases (24h)
Ituri	299	46	15.4%	15 out of 36 (41.7%)	12
North Kivu	19	1	5.3%	7 out of 34 (20.6%)	0
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	321	48	15.0%	23 out of 104 (22.1%)	12

3.2 Affected health zones

- **Ituri (15 health zones)**: Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mangala, Mongbwalu, Nizi, Nyankunde, Rwampara.
- **North Kivu (7 health zones)**: Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha.
- **South Kivu (1 health zone)**: Miti-Murhesa.
- **Newly affected (May 31, 2026)**: Logo (Ituri).

3.3 Time Analysis

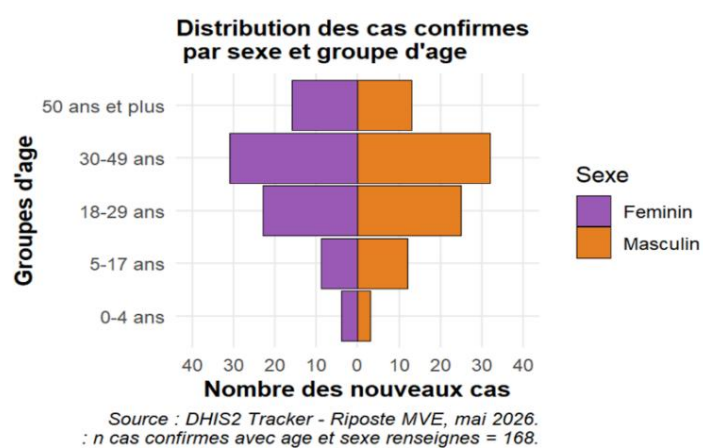


Although the data is not complete, the current evolution of the disease shows a decrease in community transmission

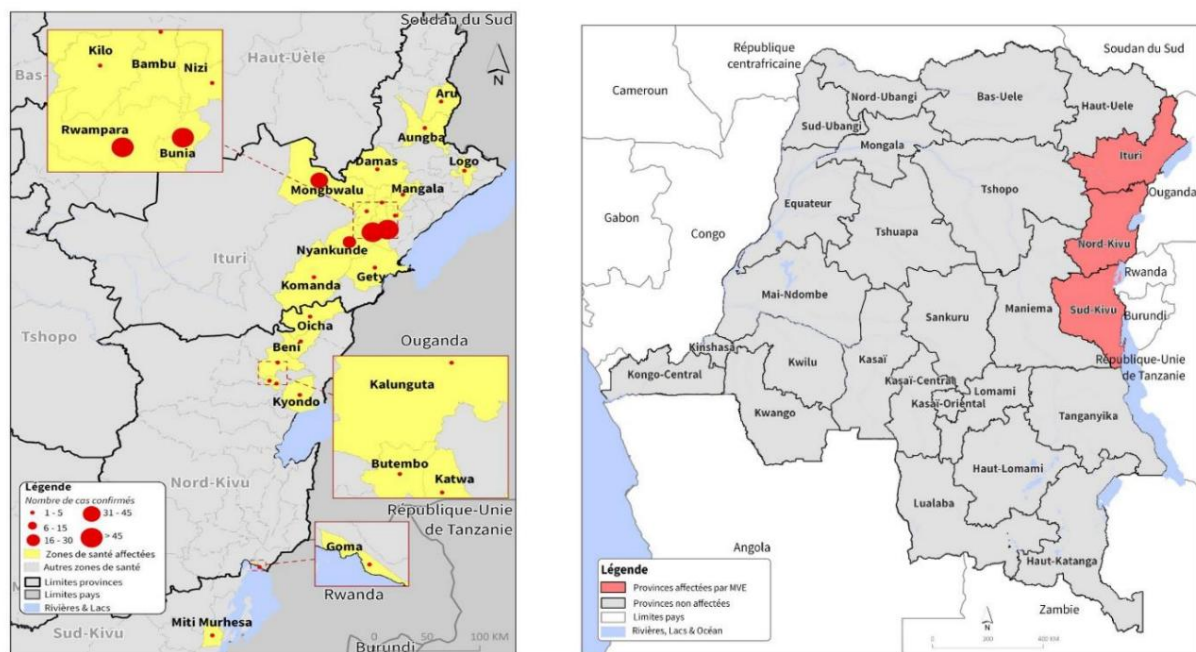
3.4 Demographic Analysis

The **18-49** age group is the most affected, with a **male predominance** among the confirmed cases (approximately 55% male).

- The extreme age groups (children aged 0 to 4 years and adults aged 50 years and over) have a lower proportion of cases, although their higher risk of lethality requires special attention.



3.5 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of May 31, 2026.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

- Organization of the discharge ceremony for four (4) patients who have recovered at the CME/Zs Rwampara, presided over by the Director General of the INSP.
- Supervised support from the Director General of INSP in the technical pillars (laboratory, planning, data management and development of SITREP).
- Visit of the coordination team to the health zones to assess the zonal approach to the riposte.
- WHO technical support to all pillars of the response.

4.2 Epidemiological surveillance

Monitoring indicators (24h)

Indicator	Value	Target	Performance
Alerts raised	192	...	...
Alerts investigated	170 (88.5%)	≥ 95%	Below the target
Alerts confirmed	117 (68.8%)	...	...
New health zones affected	1 (Logo)	0	Geographic expansion

- Continuation of surveillance activities (reporting of alerts, investigations, active search, listing and monitoring of contacts, retrospective documentary review of cases and deaths, etc.).

Challenges in surveillance

- Community resistance in the Nizi health zone to contact tracing and follow-up around confirmed cases.
- Twelve (12) alerts faced community resistance in the Bunia health zone, hindering their verification.
- Nine (9) PPL (Probable/Sampled) alerts are currently being verified in the health zone of Bunia.

Point of Entry and Point of Control (PoE/PoC) Surveillance

PoE/PoC Indicator	Value (24h) Rate	
14,057 travelers who passed through PoE/PoC	...	
Screened travelers	13,901	98.8%
Travelers who have washed their hands	13,901	98.8%
Aware travelers	13,963	99.3%
Listed contacts	0	...
Alerts raised	1	...

4.3 Laboratory

Laboratory indicators (24 hours as of May 31, 2026)

Indicator	Value Analysis	
Samples collected	65	sustained activity
Samples analyzed	18	27.7% of the collected
Positive samples	12	Positivity rate 66.6%
Samples currently being analyzed	47	Delay in analytical capacity
Positive samples from deceased suspected cases	6	Transmission before confirmed death

A bottleneck has been identified: 47 samples are awaiting analysis. Insufficient processing capacity is delaying diagnostic confirmation and the start of treatment. The arrival of RADIONE tests offers hope for improvement in the coming days.

4.4 Infection Prevention and Control (IPC)

Reported incident : Public uprising against the EDS team at the University Clinics (service providers implicated in a death). One (1) safe and dignified burial (EDS) was not carried out. Similar incident in the Logo health zone.

Actions by health zone

Health Zone Actions Taken	
Bunia	Provision and installation of waste management equipment; decontamination of 1 ESS (Marie Claire Clinic); 5 EDS carried out out of 6 alerts reported; 1 decontamination; 1 sample taken (SWAB)
Rwampara	Ring around CME Nyankunde: 3 health facilities, 9 schools, 9 churches, 1,496 households identified; safe injection briefing at CS Ngona; 3 EDS, 3 decontamination, 3 SWAB
Bamboo	Installation of 6 handwashing stations at the HGR
Aru	Donation of 4 handwashing stations to the Anglican Church

4.5 Holistic Care and Case Management

Psychosocial interventions and mental health

- Announcement of twenty-two (22) results, including 4 negative ones.
- Six (6) deaths recorded in the health zones of Rwampara, Bambu and Nyankunde.
- Two (2) additional deaths (CTE Bunia and HGR Rwampara).
- Four (4) bereaved families received psychosocial support.
- Fifty (50) families of suspected cases and twelve (12) families of confirmed cases received support psychosocial.
- Sixteen (16) patients before transfer including two critical cases (CME).
- More than 1,000 people reached by mental health interventions (military personnel, households, taxi drivers, religious leaders, students, teachers and healthcare workers).

Indicators of care

Indicator	Value
Patients in isolation	129 (all CTEs combined)
Confirmed cases in isolation	...
Suspected cases in isolation	104
Healed by the day	4
Hospital mortality	Data in progress
Available vs. Occupied CTE Beds: Evaluation in progress	

Table I. Patient movement in healthcare facilities as of May 31, 2026

ISSUES		BGRa	HGR Rwampara	CME	HGR Mongbwalu	HGR Nyankunde	BGRbu	HGR	Total
Report (Number of patients in bed at the end of the previous day)		21	15	15	12	39	10	5	117
News Today's admissions	PPL	0	1	3	1	0	0	0	5
	Others	3	5	3	5	2	2	1	21
Outings	Healed	0	0	0	0	0	0	0	0
	Deceased	1	2	4	3	1	1	0	11
	No Case	0	0	0	0	0	0	0	0
	Escapees	1	3	0	0	3	0	0	7
	Transferred to HGR	0	0	0	0	0	0	0	0
Patients in isolation	Total	23	17	17	15	38	13	6	129
	NC confirmed	3	0	2	1	0	0	0	6
	AC confirmed	1	1	1	0	11	1	1	16
	Suspects	19	15	14	14	27	10	5	104
	Suspect Child < 5 years old	0	1	0	0	0	2	0	3

4.6 Risk Communication and Community Engagement (RCCE)

Zone of health	Activity	Scope
Bunia	Handwashing awareness campaign by the Directors General of INSP and WHO; interactive program on RTNC	Large audience
Rwampara:	Awareness-raising among 190 community leaders (17 women) on preventive measures	190 people
Bamboo	Awareness-raising at Kilomoto Primary School	305 students (102 girls)
Aru	Briefing of 27 people on CREC and community-based surveillance	27 people
Nyankunde	Educational talk at Brahanam Church (72 worshippers); awareness-raising in the Sota Health Area (391 people, 233 women)	463 people

Rumors and misinformation identified

- "HGR Bunia sends people to infect community toilets to spread Ebola"
- "There's a lot of money involved in this; once admitted to the CTE, patients are injected with water to make them die."
- "What is the quality of care at the CTE?"
- "If the suspected person's results are negative, who will pay the treatment costs?"
- "Should we separate from our marital bed during this period?"

Analysis: These rumors reveal a deep mistrust of the healthcare system and response teams, a lack of understanding of treatment procedures, and legitimate economic concerns. A rumor response plan with tailored messaging must be activated.

4.7 Logistics

- Receipt of 30 motorcycles as logistical support from the central government for surveillance and coordination.

4.8 Security

The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm, but the presence of armed groups in the territories of Djugu, Irumu and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

5. MAIN CHALLENGES

Challenge No.	Impact	Action required
1. Reluctance to perform post-mortem sampling (swabs)	Delay in confirmation; underestimation of cases	Intensive community engagement; involvement of religious leaders
2. Insufficient capacity in standardized CTES despite the opening of Rwampara	Potential saturation of existing CTES	Accelerating construction planned CTES
3. Weakness in contact tracing (overall rate 43%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4. Supply failure in Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for the allocation of MEG
5. Lack of PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6. Low escalation of alerts in the 3 provinces	Delay in detection of case	Strengthening of the community monitoring
7. Spread of rumors and traditional recipes	Distrust of the response; consultation delays	Rumor response plan; social mobilization
8. Insufficient resources financial for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action		Pillar responsible	Due date
1	Training of service providers on the use of PCR RADIONE	Laboratory	Day 2
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3	Briefing of community health workers on Ebola virus disease surveillance and SBC (Ituri province)	Monitoring/CREC	Day 3
4	Official handover of the standardized CTE CME Rwampara to the Incident MVE Manager	Coordination	Day + 1
5	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
6	Catching up on contact tracing in Ituri (current rate 35%)	Monitoring	Day 7
7	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
8	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
9	Catch-up plan for the 47 samples awaiting analysis	Laboratory	Day 2

Handover by the WHO Director-General of the standardized Ebola Treatment Centre - CME Rwampara at IM MVE



Supervised support from the Director General of INSP in the various work of the pillars including the laboratory, data management and the development of SITREP with the technical support of the WHO.



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